

All figures sourced from Government of Canada and Government of Alberta open data, Statistics Canada, federal and provincial statute, UN treaty, and peer-reviewed research.

Movement Three — The Intersection Chapters

The architecture documented across Movements One and Two operates against every Albertan and every Canadian who passes through it. It does not operate identically against every population. Movement Three walks the populations within the senior and disabled-adult communities for whom the same architecture lands with specific additional structural pressures — pressures that are documented, that are disproportionate, that have causes located in identifiable policy choices, and that the federal-and-provincial system has not designed itself to address.

Each Section of this Movement extends the Movements One and Two analysis into a specific population. Each draws on the existing campaign canon where relevant and on primary-source data published by Statistics Canada, the Government of Canada, the Government of Alberta, and recognized advocacy and research organizations. None of the Sections claims that the population it documents is uniquely affected; each identifies how the architecture's operation produces outcomes for that population that differ in kind or degree from outcomes for the broader recipient pool.

Section 3.1 — Indigenous Seniors: A Senior-System Built Around an Age Most Will Not Reach

The Alberta Disability System Breakdown campaign's *Indigenous Disability in Alberta* report, published April 2026, established across 14 evidentiary sections that the federal and provincial governments of Canada have produced and sustained conditions that disable Indigenous peoples at rates substantially higher than the non-Indigenous population, and have failed to deliver the remediation their own official frameworks direct. The report's central statistical finding is that in 2023, the average life expectancy at birth for First Nations people in Alberta was 62.81 years, compared to 81.88 years for non-First Nations Albertans. The difference is 19.07 years. (Source: *Alberta Indigenous Primary Health Care Advisory Panel — Report on First Nations Life Expectancy in Alberta, 2022*; *Alberta First Nations Information Governance Centre health statistics, 2023*; *Alberta Disability System Breakdown — Indigenous Disability in Alberta Report, April 2026, Section 2.*)

This Section does not restate the 14-section *Indigenous Disability in Alberta* report. It extends the report's analysis specifically into the senior-system architecture documented across Movements One and Two of this document. It addresses one structural question that the existing report identifies and that the present document is positioned to answer: what does it mean to build a senior income system around eligibility at age 65 when the population the system is supposed to serve includes a major cohort whose average life expectancy at birth is below that threshold?

The structural finding: the senior system is built around an age most First Nations Albertans will not reach

The federal Old Age Security pension begins at age 65. The Guaranteed Income Supplement begins at age 65. The Allowance is available to spouses of OAS recipients; the Allowance for the Survivor is available to widowed survivors aged 60 to 64. The Alberta Seniors Benefit, Coverage for Seniors, Special Needs Assistance for Seniors, Dental and Optical Assistance for Seniors, and the Seniors Property Tax Deferral all begin at age 65, contingent on receipt of OAS. The Alberta long-term care accommodation framework's disposable-income guarantee applies to recipients of the senior provincial benefits.

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The architecture, as documented in Movement Two, is structured around the assumption that the recipient will reach age 65 and live for some years thereafter. Average Canadian life expectancy at birth is approximately 81.5 years, with the 65-year-old Canadian's residual life expectancy at approximately 19 to 22 years depending on sex. (Source: *Statistics Canada — Life Tables, Canada, Provinces and Territories, 2020-2022 reference period.*) The senior income system is, at the level of life-cycle math, a 16-to-22-year benefit relationship for the average Canadian.

For the average First Nations Albertan, the 65-year threshold is not reached. The 19.07-year mortality gap means that, in 2023, the average First Nations Albertan died approximately 2.19 years before the OAS eligibility threshold opened. The First Nations Albertan who would have been eligible for OAS, GIS, the Alberta Seniors Benefit, Coverage for Seniors, and the broader senior architecture statistically did not survive to access it. The federal and provincial premium they had paid into across the working years — CPP contributions, federal income tax, provincial income tax, and the rest — produced no senior-stage benefit because there was no senior stage.

This is the structural finding. It is not a critique of the senior system's design intent. It is an observation of the system's operational reality against the demographic profile of the Indigenous Albertan population. A program built around aging into benefits does not, in practice, serve a population that does not, on average, age into the benefits. The provincial government that designs the senior program inventory, the federal government that administers the OAS-CPP-GIS architecture, and the broader Canadian public-policy framework that frames senior income as a deferred-compensation arrangement all proceed as if the demographic premise applies uniformly. It does not.

What this means at the program-by-program level

Old Age Security. Indigenous people who have lived in Canada throughout their lives meet the residency requirement for the maximum OAS pension. The structural barrier is documentation. OAS application requires verification of residency periods, including periods that may have been spent in remote communities without consistent state recordkeeping, in residential schools where institutional records may have been destroyed or are difficult to access, in foster care under the Sixties Scoop where placement records are fragmentary, or in periods of pre-band-membership status changes that affect which federal records are available. Where documentation is incomplete, the application requires the applicant to assemble supplementary evidence (sworn statements, secondary records, oral history) that the federal adjudication staff may or may not accept.

The applicant who reaches age 65 and applies for OAS but cannot fully document their residency may receive a partial OAS pension, or may experience a multi-month delay while the adjudication process proceeds, or may need to file an appeal. During the delay, the applicant has no OAS income.

Canada Pension Plan retirement. The CPP retirement formula is calculated against the applicant's contribution history. Indigenous workers — particularly those in northern communities, on-reserve communities, seasonal traditional-economy work, and the broader pattern of structural underemployment documented in the Indigenous Disability in Alberta report's Sections 4-7 — frequently have CPP contribution histories that produce small retirement pensions. The general dropout provision (8 lowest-earning years excluded) and the disability dropout (months on CPP-D excluded) may apply, but they do not eliminate the impact of decades of contribution interruption.

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For a First Nations Albertan whose primary working-life income was in cash-economy or on-reserve contexts not subject to CPP withholding, the lifetime CPP contribution may be small. The CPP retirement pension calculated against that contribution is small. The senior who then qualifies for GIS finds the small CPP pension reducing the GIS amount under the standard fifty-cents-per-dollar phase-out, producing a total income near or at the GIS-only floor.

The Guaranteed Income Supplement and the partnership penalty. The GIS partnership penalty documented in Sections 2.4 and 2.7 (\$883.54/month, \$10,602.48/year for being together rather than apart at low income) operates without modification on Indigenous senior couples. The Form ISP-3040 "Living Apart for Reasons Beyond their Control" mechanism applies. Indigenous senior couples whose family arrangements include extended-family co-residence, multi-generational households, or community-based shared living may encounter administrative complications around how the GIS calculation treats the household, particularly where the federal program staff are not trained in Indigenous family-structure variations.

The CPP-D-to-AISH offset (Section 2.3). The 16,161 Albertans on AISH receiving CPP-D whose federal benefit is captured dollar-for-dollar by the provincial offset include disproportionate Indigenous representation, given the documented elevated disability rates in the Indigenous population. The CPP-D contribution that the Indigenous worker made through their working years produces zero net benefit at the working-age stage. At the conversion to CPP retirement at age 65 — for those who reach it — the federal retirement income reduces GIS at the senior stage. The capture pattern Movement Two documented operates against the Indigenous recipient at every life stage available to them.

Long-term care. Long-term care facilities on First Nations reserves in Alberta are limited. Most First Nations seniors requiring LTC access off-reserve facilities, often in urban centres distant from family, language, culture, and ceremony. The placement is consequential. The Indigenous senior in an off-reserve LTC facility is institutionalized in a setting that does not reflect their first language (where it is not English or French), does not include culturally appropriate food, does not have on-staff Elders or ceremonial space, and does not provide care in the community that the senior has, in many cases, lived in for their entire life.

The Indian Hospital system, documented in *Separate Beds: A History of Indian Hospitals in Canada, 1920s–1980s* by Maureen Lux (University of Toronto Press, 2016), operated as a parallel federal Indigenous-only institutional healthcare system through most of the 20th century. Indigenous seniors aged 75 and over today were children, adolescents, or young adults during the Indian Hospital era. Some are direct survivors of Indian Hospital placement. The institutional-care trauma of that history affects the present-day willingness of Indigenous seniors to enter LTC facilities, even when LTC is medically necessary. The 2017 *Brown v. Canada* (Attorney General) class action settled federally with respect to certain categories of survivor harm; the present-day senior-LTC policy structure does not, in operational design, account for this history.

The jurisdictional fracture at the senior stage

The Indigenous Disability in Alberta report's Section 3 documented the federal-provincial-status-non-status jurisdictional fracture that determines which institutional system is responsible for which Indigenous person at which moment. The fracture operates at the senior stage in specific ways:

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- **OAS and GIS** are federal programs. Indigenous seniors apply through Service Canada the same way non-Indigenous Canadians do. The federal program does not differentiate.
- **CPP retirement** is federal. Same.
- **Alberta Seniors Benefit, Coverage for Seniors, Special Needs Assistance for Seniors, Dental and Optical Assistance for Seniors, Seniors Property Tax Deferral, SHARP** are provincial. Indigenous seniors living off-reserve apply through Alberta Supports the same way non-Indigenous Albertans do. Indigenous seniors living on-reserve are technically eligible (provincial residency requirement is met by Alberta residency, which includes on-reserve residency), but the administrative pathway through Alberta Supports may not function smoothly for on-reserve applicants whose mailing address, banking arrangements, and identification documentation differ from off-reserve patterns.
- **Long-term care** in Alberta is provincial through Alberta Health Services. On-reserve continuing care is, in some cases, federally funded through First Nations and Inuit Health Branch (FNIHB), with delivery through community-controlled providers. The transition between on-reserve home and continuing care community placement off-reserve crosses the federal-provincial boundary. Coordination is documented in advocacy literature as inconsistent. (*Source: Indigenous Watchdog — First Nations long-term care reports, 2023-2025; Canadian Association of Retired Persons — Indigenous Senior Care Reports, 2024.*)
- **Non-Insured Health Benefits (NIHB)** is the federal program covering certain prescription drugs, dental, vision, mental health, medical transportation, and medical supplies for status First Nations and Inuit individuals. (*Source: Indigenous Services Canada — Non-Insured Health Benefits page, 2026.*) NIHB and provincial Coverage for Seniors are both potentially available to status First Nations seniors. Coordination between them is documented as inconsistent. The benefit flow depends on which program adjudicates first, what the status determination is, and how the dual-coverage rules apply at the pharmacy or service-provider level.

For the status First Nations senior, the available benefit architecture is more complex than for the non-Indigenous senior. The complexity does not produce more benefit. It produces more administrative friction. The senior who navigates federal NIHB, provincial Coverage for Seniors, and federal OAS/GIS simultaneously navigates three program structures at once, with three application processes, three appeals processes, and three coordination points where the wrong information at one program can disrupt another.

Sponsorship and the Indigenous senior

The October 2025 sponsorship-rule change documented in Sections 2.1 and 2.4 has limited application to Indigenous seniors, since Indigenous Canadians born in Canada are not subject to immigration sponsorship. The change has more direct effect on immigrant senior populations (Section 3.3 below). For Indigenous seniors with non-Indigenous spouses who are themselves immigrants, the sponsorship interaction may be relevant in specific household configurations. For most Indigenous seniors, the OAS-deferral-disqualifier and the broader Alberta Seniors Benefit threshold structure are the more directly relevant federal-provincial interaction points.

What the existing campaign canon establishes that this Section incorporates

The *Indigenous Disability in Alberta* report's 14 sections — historical context (residential schools, Indian Hospitals, Sixties Scoop), the 19-year mortality gap, the jurisdictional fracture, housing crisis, contaminated land, water crisis, healthcare racism (Brian Sinclair 2008, Joyce Echaquan 2020), Jordan's Principle, child

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welfare (72% Indigenous caseload), justice system (10.2-fold incarceration overrepresentation), MMIWG2S+, consultation failure and UNDRIP Article 21, Alberta Sovereignty Act and Treaty, and TRC Calls 18-24 — establish the structural conditions within which the senior-system architecture operates against Indigenous Albertans. (Source: *Alberta Disability System Breakdown — Indigenous Disability in Alberta Report*, April 2026.)

This Section adds, to that documented record, the senior-system-specific observation: the architecture documented in Movements One and Two of the present document is, in operational reality, not designed for the population whose average life expectancy ends before it engages. A program built around aging into benefits does not serve a population that does not age into the benefits. The administrative fixes available to address this — improved CPP contribution recognition, improved OAS application support, improved provincial seniors-benefit access for on-reserve residents, improved LTC cultural appropriateness, improved NIHB-and-Coverage for Seniors coordination — are documented in advocacy literature and have been the subject of decades of recommendation. The fixes have not been implemented at scale. The 19.07-year mortality gap continues to widen during the period of documentation. (Source: *Indigenous Watchdog — First Nations life expectancy 19 years lower than other Albertans*, February 2025.)

The lifecycle observation

For the Indigenous Albertan who lives with disability — the population at the centre of the campaign's *Indigenous Disability in Alberta* report — the working-age cascade documented in Movement One operates against the additional baseline of the federal-provincial jurisdictional fracture that structurally limits access to physicians, documentation, treatment, and supports throughout the working years. The recipient who reaches the AISH-to-ADAP transition on July 1, 2026 carries forward into the transition the cumulative documentation gaps that the fracture has produced. The Medical Review Panel reassessment process is, in operational reality, more challenging for the Indigenous recipient than for the non-Indigenous recipient with the same medical condition, because the documentary infrastructure required to satisfy the Panel is less accessible to the Indigenous applicant.

For the Indigenous Albertan who survives the working years and reaches age 65 — a smaller proportion of the population than the non-Indigenous comparator — the senior architecture engages with the same documentation friction, the same jurisdictional complexity, the same partnership penalty, the same long-term-care cultural inadequacy. The senior years that most Canadians anticipate as a deferred-compensation phase of life are, for the Indigenous senior population, an additional layer of administrative navigation against the same architecture that has operated against them across the lifecycle.

The cage that this document names operates against the Indigenous Albertan population with all of the features documented in Movements One and Two, plus the additional features documented in the *Indigenous Disability in Alberta* report. The 19-year mortality gap is, in operational reality, the cumulative biological outcome of the conditions both documents describe. The senior system that this document walks is the program inventory the surviving cohort accesses if they reach it. The cohort that does not reach it is, statistically, larger than the cohort that does.

Section 3.2 turns to 2SLGBTQ+ seniors — a population for whom the senior architecture's assumptions about family structure, household configuration, and lifecycle relationships produce specific operational gaps.